

Dental Pain Bridging Care

Patient Group Direction, version 002, issued 7 September 2026

Scope of this PGD

This PGD authorises the supply of amoxicillin as bridging antibiotic care for a clearly localised acute dental infection in an adult awaiting urgent dental assessment.

- It does not provide analgesia. Where a patient needs pain relief, offer pharmacy-sale analgesia under protocol. Where ibuprofen is unsuitable, supply paracetamol per the OTC pathway and record the reason.
- It does not treat dental infection definitively. It bridges the patient to urgent dental care, which must be arranged within 24 to 48 hours.
- **Any facial swelling, and any systemic feature, takes the patient outside this PGD. See Appendix 1.**

Amoxicillin 500mg capsules

Patient Group Direction for the supply of amoxicillin 500mg capsules as bridging care for acute dental infection in adults aged 18 years and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC. Amoxicillin is not a controlled drug, so registered pharmacy technicians may lawfully supply under this PGD (Human Medicines Regulations 2012 as amended, 26 June 2024).
- Must have completed training relevant to this condition, documented and overseen by the Get Real Health team.
- Must be competent in recognising the dental red flags set out in Appendix 1, and in particular in identifying facial swelling and airway compromise.
- Must previously have used PGDs to supply medication.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.
- All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

The PGD

Indication	Bridging antibiotic care for a clearly localised acute dental infection in an adult awaiting urgent dental assessment. This is not definitive treatment; the definitive treatment for dental infection is dental.
Inclusion criteria	• Adults aged 18 years and over. • A clearly localised dental infection: pain, tenderness, localised gum inflammation or purulent discharge confined to the tooth and adjacent gum. • NO facial swelling of any kind. • NO systemic features: temperature below 38C, and no malaise, rigors or feeling generally unwell. • Willing and able to arrange an urgent dental appointment within 24 to 48 hours. • Capable of giving valid informed consent, and consent given. • No exclusion criterion present, and no red flag from Appendix 1.
Exclusion criteria	Refer, do not supply, where any of the following is present. • ANY facial swelling, however localised or minor it appears. • Any red flag listed in Appendix 1. • Temperature of 38C or above, or malaise, rigors or systemic illness. • Under 18 years of age. Children with dental infection are referred; this PGD provides no paediatric dose, no suitable formulation and no paediatric red-flag route. • Known penicillin or beta-lactam allergy, or any history of cephalosporin allergy. • Known significant renal impairment. Refer. A five-day bridging course is not the place for a dose adjustment. • Infectious mononucleosis or acute lymphoblastic leukaemia (risk of rash). • Already taking an antibiotic for this or any other indication. • A course of amoxicillin already supplied for this episode. One supply per episode only. • Unable to give valid informed consent, or unwilling or unable to arrange dental review.
Cautions	• Concurrent SSRIs or anticoagulants: increased GI bleeding risk with NSAIDs. Relevant only where the patient is separately purchasing analgesia; note it and advise accordingly. • Amoxicillin may reduce the efficacy of oral typhoid vaccine. Not

	otherwise a barrier to supply. Advise that any rash should stop the course and prompt review.
Actions if the patient is excluded or declines	Explain why an antibiotic cannot be supplied. Arrange or advise urgent dental assessment, and where any red flag or facial swelling is present arrange same-day medical or dental assessment rather than a routine appointment. Where analgesia is appropriate and ibuprofen is unsuitable, supply paracetamol per the OTC pathway and record the reason. Document the advice given and the decision reached. Inform the GP where the reason for exclusion is a new clinical finding.

The medicine

Name, form and strength	Amoxicillin 500mg capsules.
Legal category	POM.
Route and method	Oral. Swallow whole with water. May be taken with or without food; absorption is unaffected by food.
Dose and frequency	500mg (one capsule) three times a day for 5 days.
Quantity to be supplied	15 capsules. One capsule three times a day for 5 days.
Maximum treatment period	5 days. One supply per episode. A repeat supply is not authorised under this PGD. A patient who is no better after the course requires dental assessment, not a second antibiotic.
Adverse effects	Very common: diarrhoea, nausea. Common: rash, vomiting, candidiasis. Uncommon: abdominal pain, headache. Rare but serious: anaphylaxis, Stevens-Johnson syndrome, toxic epidermal necrolysis, Clostridioides difficile infection, cholestatic jaundice. Any rash, breathing difficulty or facial or tongue swelling after a dose requires the patient to stop the medicine and seek urgent assessment.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	- That valid informed consent was given. - Patient name, address, date of birth, and the GP with whom they are registered. - Name and registration number of the healthcare professional supplying. - Name of the medicine, date of supply, dose, form, route and quantity supplied. - Batch number and expiry date. - That the patient was advised to arrange an urgent dental appointment within 24 to 48 hours, and the advice given. - Advice given if the patient is excluded or declines treatment, and the reason for exclusion. - Details of any adverse drug reactions and the actions taken. - That the medicine was supplied under this PGD. Records should be signed, dated, clear, legible and contemporaneous. Electronic records may be made. Records are kept for 8 years. This PGD excludes anyone under 18, so no paediatric retention period applies.
Storage	Store below 25C in the original packaging.

Information for the patient

Written information	Supply the amoxicillin patient information leaflet. Do not supply the ibuprofen leaflet; ibuprofen is not supplied under this PGD.
Counselling	• This antibiotic is a bridge, not a cure. You must see a dentist within 24 to 48 hours. The infection will not resolve without dental treatment. • Take one capsule three times a day for five days and finish the course. • Come back or seek urgent help the same day if your face or jaw begins to swell, if you cannot open your mouth fully, if swallowing becomes difficult, if your voice changes, or if you develop a fever. • Stop the medicine and seek urgent help if you develop a rash, wheeze, or swelling of the lips or tongue. • This antibiotic may cause diarrhoea. Seek advice if it is severe or persistent.
Follow-up	Urgent dental appointment within 24 to 48 hours. Seek advice sooner if any of the warning signs above develop, or if symptoms worsen at any point. Do not wait to finish the course before seeking help if you are getting worse.

Appendix 1 — Dental red flags

Any one of these means the patient leaves this service and is assessed the same day. Do not supply an antibiotic and send them away.

Airway and swallowing	• Trismus: unable to open the mouth fully. • Dysphagia: difficulty or pain on swallowing. • Drooling, or inability to manage saliva. • Voice change, hoarseness or a muffled "hot potato" voice. • Any difficulty breathing, stridor, or inability to lie flat.
Swelling	• ANY facial swelling. • Swelling of the floor of the mouth, or elevation of the tongue. • Swelling crossing the midline or extending into the neck. • Periorbital or orbital swelling, or any change in vision or eye movement.
Systemic	• Temperature 38C or above. • Rigors, confusion or new drowsiness. • Heart rate above 100 at rest, or respiratory rate above 22. • Systolic blood pressure below 100. • Looks unwell to you, whatever the numbers say.
Host factors	• Immunosuppression of any kind, including diabetes that is poorly controlled. • Pregnancy. • Recent head, neck or facial surgery or trauma.

Ludwig's angina and spreading odontogenic infection can progress from apparently minor swelling to airway compromise within hours. The threshold for referral in this service is deliberately low, and it is set at any swelling at all.

Authorisation

Version	v002
Supersedes	Version 001
Valid from date	7 September 2026
Expiry date	31 July 2027

Doctor	Name: Nitin Shori Job title: Medical Director, Get Real Health GMC: 6047293 Signed: N. Shori Date: 7 September 2026
Pharmacist	Name: Chris Pilkington Job title: Head Pharmacist, Get Real Health GPhC: 2046322 Signed: C. Pilkington Date: 7 September 2026

Both authorising signatories reviewed this version together on 7 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: Job title and organisation: Signature: Date:
Professional group signatory	Name: Job title and organisation: Signature: Date:

Change history

Version	v002
Date	7 September 2026
Changes	• Full clinical review and reissue.